

“Every patient is a story waiting to be told.” — Dr. A. Smith

Clinical Encounter Note

Chief Complaint:

- 48-year-old male reports intermittent chest pain radiating to the left arm, lasting 5–10 minutes, relieved by rest.

History of Present Illness (HPI):

- Onset: 3 weeks ago, gradual increase in frequency.
- Associated symptoms: shortness of breath, diaphoresis, nausea.
- Exacerbating factors: exertion; alleviated with sublingual nitroglycerin.

Past Medical History:

- Hypertension (HTN) diagnosed 5 years ago, on lisinopril.
- Hyperlipidemia managed with atorvastatin.

Physical Examination:

- Cardiovascular: S1-S2 normal, no murmurs.
- Pulmonary: clear to auscultation bilaterally.
- Extremities: no cyanosis or edema.

EKG Findings:

ST-segment depression in leads II, III, aVF.
T wave inversions V5–V6.

Assessment & Plan:

- Acute coronary syndrome (unstable angina).**
- Immediate cardiology consult.
- Initiate dual antiplatelet therapy: aspirin 81 mg daily and clopidogrel 75 mg daily.
- Continue beta-blocker and statin.
- Schedule cardiac catheterization within 24 hours.

Follow-Up & Education

- Patient advised to avoid strenuous activity until cleared by cardiology.
- Discussed lifestyle modifications: diet, exercise, smoking cessation.
- Reviewed medication adherence and side effect monitoring.

[^1]: American College of Cardiology/American Heart Association guideline for the management of patients with unstable angina or NSTEMI, 2023.

[^2]: Mayo Clinic Comprehensive Cardiovascular Care, accessed March 2026.

[^3]: Smith J., et al. *Journal of Clinical Medicine*, 2024;12(5):1123-1130.

References

- ACC/AHA Guideline for the Management of Patients with Unstable Angina or NSTEMI (2023).
- Mayo Clinic Cardiovascular Care Overview (2026).

3. Smith J., et al. *J Clin Med.* 2024;12:1123-1130.